

**GOVERNMENT OF TELANGANA
EMPLOYEES HEALTH CARE TRUST**

Present: Sri Sanjay Jaju, IAS

Proceeding No.01/EHCT/NEHS/1505/2026 Dated:22.08.2026

Sub: EHCT – New Employees Health Scheme (NEHS) – Board-approved Guidelines for Implementation of the Scheme – Orders – Issued.

Ref: 1. G.O.Ms.No.40, HM&FW (A) Dept., dt.17.06.2026.
2. G.O.Ms.No.50, HM&FW (A) Dept., dt.16.07.2026.
3. Minutes of the Meeting No. 03/EHCT/NEHS/2026, Dated:31.07.2026.

-oOo-

Order:

In pursuance of the Government Orders cited and with the approval of the Board of Trustees, the comprehensive "Guidelines for Implementation of the New Employees Health Scheme (NEHS)" framed for effective implementation, administration and management of the Scheme are hereby issued for implementation by all concerned. The said Guidelines are appended to these proceedings and shall form part of these proceedings.

The Guidelines shall come into force with effect from 17.07.2026. The Chief Executive Officer, EHCT, and the authorities specified in the Guidelines shall take necessary action for implementation accordingly.

**Sd/-
Sri Sanjay Jaju, IAS
Chief Secretary to Government
& Chairman of EHCT, Telangana**

//Attested//



Member Convener & CEO, EHCT

To:

1. The Special Chief Secretary/Principal Secretary/Secretary of all departments, Secretariat, Telangana.
2. All the HoDs concerned.
3. Stock File / Spare.

GUIDELINES FOR IMPLEMENTATION OF THE NEW EMPLOYEES HEALTH SCHEME (NEHS)

**(To be appended to Proceedings No. 01/EHCT/NEHS/1505/2026,
dated 22.08.2026)**

PREAMBLE

The Government of Telangana introduced the New Employees Health Scheme (NEHS) to provide cashless and comprehensive healthcare services to serving State Government employees, service pensioners, family pensioners and their eligible dependants through the Employees Health Care Trust (EHCT). The framework for administration of the Scheme was prescribed, inter alia, in G.O.Ms.Nos.174, 175 and 176, dated 01.11.2013; G.O.Ms.No.32, dated 03.11.2014; G.O.Ms.No.36, dated 27.11.2014; G.O.Ms.Nos.97 and 98, dated 07.06.2017; and G.O.Ms.No.186, dated 08.10.2023.

In G.O.Ms.No.40, Health, Medical and Family Welfare (A) Department, dated 17.06.2026, the Government reconstituted the Board of Trustees of the Employees Health Care Trust and directed the Board to frame comprehensive guidelines for implementation of the New Employees Health Scheme, after taking into consideration the existing guidelines, and to propose modalities for issuance of Employees Health Cards.

Further, in G.O.Ms.No.50, Health, Medical and Family Welfare (A) Department, dated 16.07.2026, the Board of Trustees was entrusted with the powers and responsibilities necessary for effective implementation, administration and management of the Scheme. Accordingly, in pursuance of the said Government Orders, the following guidelines are issued to provide a comprehensive framework for implementation, administration and operation of the New Employees Health Scheme in a transparent, accountable and efficient manner.

TABLE OF CONTENTS

1. COVERAGE	3
2. OPERATIONAL GUIDELINES.....	3
3. SCOPE OF BENEFITS	4
4. OUT-PATIENT TREATMENT	4
5. IN-PATIENT TREATMENT	6
6. FINANCIAL COVERAGE.....	7
7. HOSPITAL ROOM ENTITLEMENT	8
8. IMPLEMENTATION MECHANISM	8
9. ESTABLISHMENT AND OPERATIONALISATION OF WELLNESS CENTRES	9
10. HOSPITAL EMPANELMENT.....	10
11. HOSPITAL PAYMENTS	11
12. DEPLOYMENT OF INFORMATION TECHNOLOGY INFRASTRUCTURE ..	12
13. MEDICAL AUDIT, FINANCIAL AUDIT AND ACCOUNTS	12
14. FINANCING AND BUDGET	12
15. GRIEVANCE REDRESSAL	14
16. GENERAL	14

1. COVERAGE

1.1 The coverage of beneficiaries under the New Employees Health Scheme is as follows:

a) Serving employees:

1. All regular State Government employees. A State Government employee will have the meaning as defined under Fundamental Rules.
2. Provincialized employees of local bodies.
3. Aarogya Bhadratha of Police Department and Aarogya Sahayatha of Prohibition & Excise Department.

b) Retired employees:

1. All Service Pensioners
2. Family pensioners
3. Re-employed service pensioners

1.2 The Scheme shall cover eligible serving State Government employees, service pensioners, family pensioners and their eligible dependant family members.

1.3 The categories of beneficiaries, constitution of family, dependency criteria and exclusions shall continue to be governed by the existing Government Orders, unless otherwise specifically ordered by EHCT.

1.4 The inclusion of beneficiaries under the Aarogya Bhadratha Scheme of the Police Department and the Aarogya Sahayatha Scheme of the Prohibition & Excise Department in the New Employees Health Scheme shall be subject to the implementation modalities to be approved by the EHCT and the Finance Department.

2. OPERATIONAL GUIDELINES

2.1 The procedure for enrolment of beneficiaries and the responsibilities of the authorities and beneficiaries concerned shall be as follows:

2.1.1 Digital Enrolment and IFMIS Validation

- (a) EHCT shall establish and maintain an online beneficiary management module as part of the EHS software platform and mobile application.
- (b) Beneficiary and dependant particulars shall be enrolled and validated through IFMIS and integrated with EHCT.
- (c) In respect of newly included institutions or categories of employees whose particulars are not available in the database, the Finance Department shall provide digitally authenticated employee data to EHCT through IFMIS integration.
- (d) Employees, service pensioners and family pensioners shall verify, update or submit their particulars in the IFMIS through the Drawing and Disbursing Officer (DDO), Pension Payment Officer (PPO) or Sub-Treasury Officer (STO) concerned, as the case may be.

- (e) Where the prescribed contribution has not been deducted because an employee draws pay outside the standard DDO/IFMIS payroll process, including in the case of a Fully Self-Drawing Officer, or for any similar administrative reason, the Drawing and Disbursing Officer or Head of Office concerned shall regularise the pending deduction in the manner prescribed by the Finance Department before activation or continuance of Health Card benefits.
- (f) Drawing and Disbursing Officers, Pension Payment Officers, Sub-Treasury Officers and departmental nodal officers shall verify and resolve exceptional, unmatched or disputed cases through IFMIS.

2.1.2 Health Cards

- (a) New EHS Health Cards shall be generated digitally and made available through the NEHS mobile application and web portal upon completion of the validation and certification of the beneficiary database through IFMIS.
- (b) The concerned DDOs, PPOs, STOs, and Departments shall ensure the verification and updating of beneficiary data through IFMIS on priority to facilitate the timely issuance of the New EHS Health Cards.
- (c) The digital Health Card, QR code, or such other secure credential as may be notified by EHCT from time to time shall be accepted by all empanelled hospitals and Wellness Centres for the identification and authentication of beneficiaries.
- (d) Until the New EHS Health Cards are issued, treatment shall continue to be provided on the basis of the Employee /Pensioner Identity number as applicable. No eligible beneficiary shall be denied admission or treatment solely on the ground that the New EHS Health Card has not yet been issued.

2.1.3 Data Correction and Family Mapping

- (a) The Finance Department shall prescribe a time-bound online procedure for correction of demographic particulars, addition or deletion of eligible dependants, submission of dual-spouse declarations and resolution of mismatches in IFMIS.
- (b) Every addition, deletion or modification shall carry a verifiable audit trail by Drawing and Disbursing Officers, Pension Payment Officers, Sub-Treasury Officers and concerned department officials through IFMIS and should be integrated with NEHS portal through API integration.

3. SCOPE OF BENEFITS

3.1 The Scheme shall provide cashless outpatient treatment, basic investigations and medicines through Wellness Centres and cashless inpatient treatment through empanelled Government and private network hospitals.

4. OUT-PATIENT TREATMENT

4.1 Out-patient treatment shall ordinarily be provided through Wellness Centres.

4.2 Consultation and treatment shall be provided for common ailments; ophthalmic, dental, ear, nose and throat (ENT), and paediatric conditions; physiotherapy; and specified long-term or chronic diseases, including diabetes mellitus, hypertension, coronary artery disease, cerebrovascular disease, cancer, renal failure, tuberculosis, leprosy, rheumatological disorders, arthritis, systemic lupus erythematosus, autoimmune diseases, urological and orthopaedic conditions, and such other ailments as require short-term or long-term sustained medical treatment.

4.3 Investigations at Wellness Centres

- (a) Haematology and Pathology: Blood grouping and Rh typing, Haemoglobin, PCV, RBC indices, Total WBC Count, Differential Count, ESR, Platelet Count, Peripheral Smear, Complete Urine Examination, Stool Examination and Pap Smear.
- (b) Biochemistry: Glucose Tolerance Test, Serum Creatinine, Urea, Uric Acid, Lipid Profile, Liver Function Tests, Prostate-Specific Antigen where clinically indicated and Serum Calcium.
- (c) Microbiology and Serology: HIV screening, HBsAg, HCV and such other tests as may be notified, subject to applicable protocols.
- (d) Radiology and other diagnostics: X-ray, ultrasonography, ECG, mammography, pulmonary function test, gynaecological investigations, diet counselling, pre-operative and post-operative investigations, and such other routine or high-end investigations as may be required.
- (e) Eye Examination: Visual acuity, refraction, intraocular pressure and fundoscopy.

4.4 Pharmacy

4.4.1 All prescribed medicines, surgical consumables, supportive accessories, emergency drugs and life-saving medicines required for common ailments and long-term treatment shall be provided through Wellness Centres in accordance with the approved formulary and procurement procedure.

4.4.2 The medicines may include those required for ophthalmic, dental, ENT, paediatric, diabetic, hypertensive, cardiac, cerebrovascular, renal, pre- and post-renal transplantation, psychiatric, hormonal, oncological, gastrointestinal, neurological, rheumatological, autoimmune, urological and orthopaedic conditions, and such other conditions as may be approved.

4.5 Referrals by Wellness Centres

4.5.1 Beneficiaries requiring investigations not available at a Wellness Centre may be referred to Government diagnostic facilities under a cashless referral mechanism.

4.5.2 Cases requiring inpatient treatment may be referred from the Wellness Centre to an empanelled network hospital by issuing a physical or electronic referral containing the provisional diagnosis and available clinical records.

4.6 Emergency Direct Admission

4.6.1 In an emergency, a beneficiary may be admitted directly to the nearest empanelled network hospital without a prior referral. Such admission shall be intimated to EHCT immediately and, in any case, not later than 48 hours from the time of admission.

5. IN-PATIENT TREATMENT

5.1 The Scheme shall provide end-to-end cashless inpatient treatment through empanelled Government and private network hospitals for approved medical, surgical and day-care procedures.

5.2 The NEHS adopted the Central Government Health Scheme (CGHS) packages and package rates notified in October 2025, as approved by the Board of Trustees.

5.3 The adoption of CGHS 1,816 packages shall not extend to outpatient services and standalone investigations. Investigations forming an integral part of an approved inpatient or day-care package shall be included in the package and shall not be charged separately.

5.4 An approved package shall include all services, medicines, consumables, investigations, implants or devices, and follow-up care specified in the corresponding CGHS package. No empanelled hospital shall demand or collect any additional amount from a beneficiary in respect of any item included in the approved package.

5.5 Where a procedure or treatment is not included in the adopted CGHS package list, the Chief Executive Officer, EHCT, shall place the matter before the competent Committee or the Board of Trustees, as the case may be, for adoption of an equivalent package, fixation of a rate or approval of an appropriate payment mechanism.

5.6 Categorisation of Hospitals

5.6.1 For the purposes of empanelment and application of package rates, hospitals may be categorised as follows:

- a) Non-NABH Hospitals
- b) NABH Hospitals.
- c) NABH Super-Specialty Hospitals.

5.7 Utilisation Management

5.7.1 In respect of medical procedures or medical management cases, treatment-progress checkpoints may be prescribed in the software at package-value thresholds of ₹3 lakh, ₹10 lakh, and, for packages exceeding ₹10 lakh, at such higher thresholds as may be specified, for utilisation review and cost containment.

5.8 Medical Management Cases

5.8.1 In medical management cases, telephonic intimation to be given to EHCT within 24 hours, subject to the treating hospital uploading the prescribed pre-authorisation particulars and clinical records subsequently.

5.9 Beneficiary Liability

5.9.1 A beneficiary shall not be required to pay for any service or item included in the approved package. Where the beneficiary voluntarily opts for accommodation higher than the entitled room category, or for a service or item not admissible under the package, the applicable differential amount shall be borne by the beneficiary.

5.9.2 A beneficiary shall pay the charges for OPD consultations and investigations at the CGHS package rates applicable at the empanelled hospitals.

5.10 Patient Facilitation

5.10.1 The field staff designated for EHS shall guide and facilitate beneficiaries from the time of registration until discharge.

5.10.2 Every network hospital shall nominate an EHS Liaison Assistant or Nodal Officer to coordinate treatment and related services for EHS beneficiaries.

5.11 Treatment Outside the State

5.11.1 Prior approval of EHCT shall ordinarily be obtained by submitting the treatment plan and estimate where the required treatment is not available within the State.

5.11.2 In an emergency occurring outside the State, the beneficiary may be admitted to the nearest appropriate hospital and shall intimate EHCT within 24 hours and, in any case, not later than 48 hours. The beneficiary shall submit the emergency certificate, essentiality certificate, discharge summary, original invoices and such other records as may be prescribed.

5.11.3 Payment shall be regulated according to the corresponding adopted package rate or the recommendation of the competent Technical Committee in respect of unlisted medical treatment.

5.12 Irregularities or Misuse

5.12.1 Any irregularity, false declaration or misuse on the part of a beneficiary shall render the beneficiary liable to recovery of the amount involved, disciplinary action under the applicable rules and penal action in accordance with law.

6. FINANCIAL COVERAGE

6.1 The employees, pensioners and their dependant family members will be eligible for a sum of Rs.3.00 lakhs (Rupees three lakhs only) per episode of illness with no limit on the number of episodes. However, in the cases where the cost of treatment exceeds Rs.3.00 lakhs, cashless treatment will continue. No network hospital shall deny the treatment in such cases.

6.2 The limit of Rs.3.00 lakhs will not apply in cases where predetermined package rates are above Rs.3.00 lakhs.

6.3 Preauth/Claims up to Rs.3.00 lakhs (Rupees three lakhs only) per treatment episode shall be reviewed and approved by the Technical Committee.

6.4 Preauth/Claims above Rs.3.00 lakhs and up to Rs.10.00 lakhs per treatment episode shall be examined and approved by a Sub-Committee consisting of the Technical Committee along with three to four Members of the Board of Trustees.

6.5 Preauth/Claims exceeding Rs.10.00 lakhs per treatment episode shall be placed before the quorum of Board/Trust in an Emergency Meeting, convened either physically or through digital mode, for appropriate decision.

7. HOSPITAL ROOM ENTITLEMENT

7.1 For the purpose of administration of the Scheme, beneficiaries shall be classified into the following Slabs based on their respective Pay Grades:

Pay Grade	Slab	Accommodation Eligibility
I to IV	Slab-A	Eligible for Semi-Private Ward
V to XVII	Slab-B	
XVIII to XXXII	Slab-C	Eligible for Private Ward

The applicable Slab in respect of Service Pensioners and Family Pensioners shall be determined with reference to the present Pay Grade of the post from which the pensioner retired from service. The hospital room entitlement of beneficiaries under the Scheme shall be governed by the existing Government Orders relating to accommodation eligibility based on pay scale, pay grade or cadre, as amended from time to time.

7.2 The entitlement of eligible dependants shall be the same as that of the principal employee or pensioner through whom they are enrolled.

7.3 Where the beneficiary voluntarily opts for accommodation higher than the entitled category, the difference in room rent and any consequential additional charges arising solely from such higher accommodation shall be borne by the beneficiary.

8. IMPLEMENTATION MECHANISM

8.1 The New Employees Health Scheme shall be implemented, administered, managed and operated by the Employees Health Care Trust under the supervision and control of the Board of Trustees.

8.2 The Board of Trustees shall exercise all powers and discharge the responsibilities entrusted under the Trust Deed, Rules and Government Orders for effective implementation of the Scheme.

8.3 The Chief Executive Officer, EHCT, shall have executive control over implementation of the Scheme and shall exercise such administrative, operational and financial powers as may be delegated by the Board of Trustees and Government.

8.4 The Trust Deed, Rules or bye-laws may, wherever necessary, be amended to give effect to these guidelines.

8.5 Establishment and Functioning of the Head Office

8.5.1 For establishment and effective functioning of the Head Office, EHCT may:

- (a) Engage staff on deputation, contract or outsourcing basis with the required skills and experience for administration, operations, pre-authorisation, claims processing, medical audit, vigilance, grievance redressal, finance and information technology, within the sanctioned strength and approved budget.
- (b) Utilise the services of field staff of Rajiv Aarogyasri Health Care Trust or other Government health institutions under arrangements approved by the competent authority.
- (c) Engage specialist doctors for pre-authorisation, claims processing and medical review on a per-case or other approved basis.
- (d) Utilise an approved service provider for information technology and supporting services.
- (e) Appoint nodal officers to coordinate with stakeholders and redress beneficiary grievances.
- (f) Procure the infrastructure, equipment and services required for implementation of the Scheme in accordance with the applicable financial and procurement rules.

8.6 Expert Committee and Executive Committee

8.6.1 EHCT shall constitute the following Committees for effective implementation of the Scheme:

- (a) an Expert Committee to review implementation issues arising under the Scheme and recommend improvements in the Scheme; and
- (b) an Executive Committee to discuss routine operational matters, coordinate with Hospital Associations and other stakeholders, and place important policy matters before the Board of Trustees for final decision.

8.7 The Trust shall ensure smooth implementation of the Scheme while protecting the interests of beneficiaries and maintaining financial sustainability.

9. ESTABLISHMENT AND OPERATIONALISATION OF WELLNESS CENTRES

9.1 The existing twelve Employees/Journalists Health Scheme Wellness Centres operationalised under G.O.Ms.No.204, HM&FW Department, dated 06.11.2025, shall continue to function and provide outpatient services to EHS beneficiaries.

9.2 Twenty-five additional Wellness Centres shall be established, in addition to the existing twelve Wellness Centres, thereby increasing the total number of Wellness Centres to thirty-seven.

9.3 The Wellness Centres shall be located so as to ensure access to services in all thirty-three districts of Telangana. The district-wise locations shall be finalised on the basis of beneficiary concentration, geographical accessibility, availability of Government infrastructure, patient load and administrative feasibility, and shall be placed before the Board of Trustees for approval.

9.4 The additional Wellness Centres shall, as far as practicable, be established in Government General Hospitals, Government Teaching Hospitals, District Hospitals, Area Hospitals or other suitable and conveniently accessible Government

premises. Where Government premises are not readily available, any other suitable location may be considered with the approval of the competent authority.

9.5 The new Wellness Centres may be placed under the functional control of the Director of Medical Education or the Director, NIMS, having regard to their location, availability of specialists and administrative feasibility, in accordance with the convergence model prescribed in G.O.Ms.No.204.

9.6 Medical Officers, specialists, nursing personnel and paramedical personnel shall be deployed by the Director of Medical Education or NIMS, as applicable. Administrative and non-medical personnel, including Data Entry Operators, office support staff, housekeeping personnel and security personnel, shall be arranged by EHCT or through such other approved mechanism as may be prescribed.

9.7 Specialists in General Medicine, Gynaecology, Orthopaedics, ENT, Ophthalmology, Dermatology, Dental, Physiotherapy and other required specialties may be deployed by DME/NIMS according to beneficiary load and availability.

9.8 Required personnel may be engaged on deputation, contract or outsourcing basis where regular personnel are not available, subject to approval of the competent authority.

9.9 Laboratory equipment, furniture and other infrastructure shall be procured through TGMSIDC/NIMS rate contracts, the GeM, approved institutional rate contracts or any other procurement method permitted under the applicable financial rules.

9.10 Medicines, drugs, surgical items, consumables and supportive accessories shall be procured through NIMS/CGHS rate contracts, approved empanelled suppliers or other permissible procurement methods.

9.11 An Internal Committee shall assess the requirement of medicines, consumables, reagents, equipment and other items and recommend procurement based on patient load and consumption.

9.12 All Wellness Centres shall, as far as practicable, follow uniform standards in respect of specialties, staffing, equipment, pharmacy, investigations, beneficiary registration, referral and service delivery.

9.13 The Chief Executive Officer, EHCT, shall prepare a phase-wise plan indicating the location, infrastructure, manpower, equipment and financial requirement of each additional Wellness Centre and place the same before the Board of Trustees.

10. HOSPITAL EMPANELMENT

10.1 The guidelines for empanelment of hospitals within and outside the State, pre-authorisation and related modalities shall conform to the standard procedures followed under the Aarogyasri Scheme, subject to such modifications as may be approved for NEHS.

10.2 Hospitals shall be empanelled having regard to infrastructure, availability of qualified specialists, nursing and paramedical personnel, equipment, diagnostic

facilities, accreditation status, geographical necessity, quality standards and patient safety requirements.

10.3 An empanelled hospital found to have committed any irregularity or misuse shall be liable to recovery of the amount involved, imposition of penalty, suspension, de-empanelment, blacklisting and criminal or other legal action, as applicable.

10.4 The list of empanelled hospitals shall be made available on the official website, mobile application and other communication channels of EHCT.

10.5 The grading of hospitals and application of approved package rates shall be determined with reference to accreditation status, prescribed standards and quality of services.

10.6 EHCT shall enter into service agreements with network hospitals specifying package rates, cashless treatment obligations, room entitlement, quality standards, claims requirements, audit rights and penalties.

10.7 For the immediate implementation of the Scheme, the network hospitals empaneled under EHS for the existing specialties shall be deemed to be empaneled under NEHS until the expiry of their current validity period.

10.8 The network hospitals empanelled under the Directorate of Medical Education (DME) for the purpose of Medical Reimbursement shall be deemed to be empanelled under the NEHS until the expiry of their current empanelment validity period.

10.9 The renewal of the empanelment of network hospitals under the NEHS shall be carried out in accordance with the Standard Operating Procedures (SOPs) followed under the Aarogyasri Scheme, with suitable modifications applicable to the NEHS.

10.10 The Chief Executive Officer, EHCT, may empanel or de-empanel network hospitals and private nursing homes in conformity with the approved standard procedure and delegated powers.

11. HOSPITAL PAYMENTS

11.1 Payments under the Scheme shall be based on the CGHS package rates notified in October 2025, as adopted by the Board of Trustees, and such other rates as may be specifically approved under these guidelines.

11.2 Where Government has notified ceiling or control prices for implants, stents, devices, drugs or consumables, the package amount shall be regulated accordingly.

11.3 All admissible payments shall ordinarily be processed on a first-in, first-out (FIFO) basis, subject to completion of medical and financial scrutiny, availability of funds, and withholding of disputed or suspected claims.

11.4 The approved package rates may be reviewed when CGHS rates are revised or when required on account of Scheme experience, subject to approval of the Board of Trustees or Government, as applicable.

11.5 The package category applicable to a hospital may be revised upon upgradation or change in accreditation status in accordance with the approved procedure.

11.6 Claims shall be supported by beneficiary authentication, case sheet, discharge summary, investigation reports, procedure notes, implant details, invoices and other records prescribed by EHCT.

12. DEPLOYMENT OF INFORMATION TECHNOLOGY INFRASTRUCTURE

12.1 The services of the existing information technology service provider of RAHCT may be utilised for implementation of the Scheme.

12.2 The new software, website and mobile application shall be utilised and suitably customised. Subject to the approved procurement and financial procedures, the Chief Executive Officer, EHCT, may develop such additional modules as may be required for beneficiary enrolment, Health Cards, Wellness Centres, hospital empanelment, pre-authorisation, claims, payments, contributions, audit and grievance redressal.

12.3 Software applications shall be integrated, wherever required, with IFMIS, Treasuries, Pension Payment Offices and other Government databases.

13. MEDICAL AUDIT, FINANCIAL AUDIT AND ACCOUNTS

13.1 Medical Audit and Financial Audit shall be conducted periodically under the supervision of the Chief Executive Officer, EHCT.

13.2 Medical Audit shall examine medical necessity, appropriateness of admission and treatment, length of stay, clinical documentation, use of implants and devices, readmissions, mortality, quality of care and adherence to package protocols.

13.3 Internal Financial Audit shall examine package-rate compliance, duplicate or irregular claims, contribution receipts, matching Government contribution, hospital payments, procurement, administrative expenditure and reconciliation of the Trust Fund.

13.4 The annual accounts of the Trust Fund shall be maintained separately and shall be audited by the Comptroller and Auditor General of India or by an agency authorised by Government, as applicable. The audited accounts, together with the audit report, shall be placed before the Board of Trustees.

14. FINANCING AND BUDGET

14.1 Financing of the Scheme

14.1.1 The New Employees Health Scheme shall be financed through monthly beneficiary contributions recovered from serving employees, service pensioners and family pensioners, an equal matching contribution by the State Government and such additional budgetary support as may be provided by Government from time to time.

14.2 Beneficiary Contributions

14.2.1 Every serving employee shall contribute 1.5% (one and one-half per cent) of the Basic Pay drawn for the month.

14.2.2 Every service pensioner and family pensioner shall contribute 1.5% (one and one-half per cent) of the Basic Pension drawn for the month.

14.2.3 The Drawing and Disbursing Officers, Pension Payment Officers, Sub-Treasury Officers, Pay and Accounts Office and Treasuries shall recover the prescribed contribution at source before disbursement of salary or pension.

14.2.4 Where both spouses are serving State Government employees, service pensioners or family pensioners, contribution by one spouse alone shall be sufficient, subject to submission of the prescribed declaration and identification particulars in the manner notified by Government.

14.3 Government Matching Contribution

14.3.1 The State Government shall provide an equal matching contribution corresponding to the aggregate beneficiary contribution collected every month. The Finance Department shall make the necessary budget provision and ensure timely release of the matching contribution to the designated EHCT Trust Fund Account.

14.4 Direct Transfer of Scheme Funds

14.4.1 The Finance Department shall ensure that the monthly funds pertaining to the New Employees Health Scheme, including the Government's contribution, are transferred directly to the designated EHCT Trust Fund Account on a monthly basis.

14.5 Trust Fund

14.5.1 EHCT shall maintain a dedicated Trust Fund Account in a Scheduled Bank approved by Government.

14.5.2 All beneficiary contributions, matching Government contributions shall be credited directly to the Trust Fund Account.

14.5.3 The Trust Fund shall be utilised exclusively for settlement of admissible claims and for operational and administrative expenditure of the Scheme as approved by the Board of Trustees, subject to the Trust Rules, Government Orders and applicable financial rules.

14.5.4 No amount from the Trust Fund shall be diverted for any purpose unconnected with the New Employees Health Scheme.

14.6 Administrative Support

14.6.1 Government shall provide a separate administrative budget for Wellness Centre operations and expansion as approved by Board.

14.7 Financial Powers

14.7.1 The Chief Executive Officer, EHCT, shall utilise the Trust Fund and Government budget in accordance with the Trust Rules, Government financial and

procurement rules, delegation of financial powers and decisions of the Board of Trustees.

14.7.2 The Board of Trustees shall approve the annual budget, hospital claims payment plan, operational and administrative expenditure, procurement plans and delegation of financial powers required for implementation of the Scheme.

15. GRIEVANCE REDRESSAL

15.1 EHCT shall establish and maintain a grievance redressal mechanism for beneficiaries and hospitals at the State, district, Wellness Centre and network hospital levels.

15.2 Grievances relating to Health Cards, beneficiary eligibility, denial of treatment, demand for payment, room entitlement, medicines, investigations, pre-authorisation, hospital coordination and quality of care shall be registered and disposed of within the prescribed timelines.

15.3 Grievances relating to Hospital pre-authorizations, claims and any other grievances shall be registered and disposed of within the prescribed timelines.

15.4 Cases involving denial of emergency treatment, unauthorised collection of money or patient safety concerns shall be examined on priority.

16. GENERAL

16.1 The EHCT may constitute one or more Committees to develop and recommend detailed operational instructions, Standard Operating Procedures (SOPs), package manuals, medical protocols, treatment guidelines, claims processing procedures, beneficiary guidance, and other operational documents required for the effective implementation of these Guidelines. Such operational documents, and any amendments thereto, may be issued with the approval of the Board of Trustees, provided they are consistent with the provisions of these Guidelines.

16.2 The Director of Treasuries and Accounts, Pay and Accounts Officer, Heads of Departments, Drawing and Disbursing Officers, Pension Payment Officers, District Collectors and other authorities concerned shall take necessary action for implementation of these guidelines.

16.3 These guidelines shall come into force with effect from 17.07.2026, as notified by the EHCT.

Sd/-

**Sri Sanjay Jaju, IAS
Chief Secretary to Government
& Chairman of EHCT, Telangana**

//Attested//



Member Convener & CEO, EHCT